

Neurologic and Bone Complications

Neurologic disease has been reported in patients with isolated celiac disease, including epilepsy, ataxia, and dementia; however, confirmation of these findings awaits large epidemiologic studies. Some authors have proposed that patients with dermatitis herpetiformis (DH) may be at higher risk for these neurologic complications due to long-standing gluten ingestion; however, Wills and co-workers found no evidence of immune-mediated neurologic disease in their evaluation of patients with DH.

Patients with untreated celiac disease have an increased frequency of bone loss. Patients with DH often continue on gluten-containing diets, leading to chronic, albeit low-grade, malabsorption. Di Stefano demonstrated significantly reduced bone mineral density in patients with DH who consume gluten. These findings suggest that patients with DH should be closely monitored, and those on gluten-containing diets should be screened for decreased bone density. If reduced bone mineral density is detected, patients should be encouraged to adopt a gluten-free diet.

Differential Diagnosis

DH may be confused with numerous other conditions due to its pleomorphic manifestations and occasional absence of diagnostic lesions.

Differential Diagnosis of Dermatitis Herpetiformis

Consider

- Eczema
- Atopic dermatitis
- Papular urticaria
- Neurotic excoriations
- Bullous pemphigoid
- Pemphigoid gestationis

- Linear immunoglobulin A dermatosis

Rule Out

- Scabies

Conditions such as eczema, papular urticaria, neurotic excoriations, pemphigoid, pemphigoid gestationis, erythema multiforme, and other bullous or inflammatory dermatoses can typically be differentiated based on histologic and immunologic findings. Linear IgA disease may resemble DH clinically and histologically but is distinct immunologically. A high index of suspicion is essential, as DH can be diagnosed even in the absence of primary skin lesions by identifying characteristic granular IgA deposits in perilesional, normal-appearing skin.

Treatment

Sulfones

Diaminodiphenyl sulfone (dapsone), sulfoxone (not available in the United States), and sulfapyridine provide rapid improvement in symptoms and signs of DH. Symptom relief may occur within 3 hours to a few days after the first dose, and new lesion formation typically ceases within 1–2 days of starting treatment. Exacerbations can occur within hours to days of stopping therapy. This rapid response was historically a key diagnostic clue.

The preferred treatment for adults is dapsone at an initial dose of 100–150 mg/day (usually administered once daily). Some patients may require up to 300–400 mg/day for initial control. Patients should be instructed to use the lowest effective dose to control symptoms. Not all patients require daily dosing; in rare cases, 25 mg weekly may suffice.

Sulfapyridine (1.0–1.5 g/day) is an alternative for patients intolerant to dapsone, elderly individuals, or those with cardiopulmonary comorbidities. Dapsone pharmacology, mechanisms of action, adverse effects, and monitoring are detailed in Chapter 226. It is important to note that nonsteroidal anti-

inflammatory drugs (NSAIDs) can exacerbate DH, even in patients taking dapsons.

Gluten-Free Diet

Effect on the Small Intestine

The intestinal lesion in DH responds to gluten withdrawal, with a time course of improvement similar to that seen in celiac disease.

Effect on the Skin Disease

Strict adherence to a gluten-free diet leads to a reduction or complete discontinuation of medication in most (but not all) patients, typically after 5 months to 1 year. These findings confirm earlier results from the study by Fry et al. and have been supported by subsequent research.